



Community Mental Health Services Triage to Discharge SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD) including all youth, adult, older adult Community Mental Health Services (CMHS) and Statewide and Specialty Community Mental Health Services (Excluding Crisis Response Services)	Medical, Nursing, Allied Health, Clerical

Purpose

This standard operational procedure (SOP) outlines the minimum mandatory requirements and provides guidance across RPBG regarding the CMHS referral and triage to discharge pathway.

Clinicians are to use informed judgement whilst applying relevant knowledge and skills to support consumers through the triage to discharge process and practice within the overarching guidance in the following documents:

- [Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [Australian Mental Health Outcomes and Classification Network - National Outcomes and Casemix Collection \(NOCC\) Technical Specifications](#)
- WA Department of Health Statewide Standardised Clinical Documentation (SSCD) for Mental Health Services Policy
- WA Department of Health SSCD for Mental Health Services Procedure
- WA Department of Health Triage to Discharge: Mental Health Framework for SSCD
- East Metropolitan Health Service (EMHS) Care Coordination in Mental Health Services Policy
- EMHS Discharge Communications Policy
- EMHS Discharge Policy
- RPBG Clinical Care of People Who May Be Suicidal Policy

Note for this document personal support persons (PSPs) includes next of kin (NoK), family, carers, an Office Public Advocate guardian or enduring guardian or a nominated person and will be referred to as "PSP."

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Version: 4
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Definitions and abbreviations

Activation/ Admission	Agreement between the CMHS and consumer to provide and receive treatment and activation on the WebPSOLIS database
ATT	Assessment and Treatment Team
CC	Care Coordinator
CMHS	Community Mental Health Services
TSDP*	Treatment, Support and Discharge Plan is a consumer care plan (for SSCD) or Stay Strong Plan (Wungen Kartup) includes Crisis or Safety Plan
CTS*	<i>Care Transfer Summary</i> (SSCD)
CTT	Clinical Treatment Team
GP	General practitioner
HoNOS	Health of the National Outcome Scales (mandatory)
K10+	Kessler 10+ a consumer self-rated measure (mandatory)
LSP-16	Life Skills Profile-16 (mandatory)
MDT	Multidisciplinary team
MHAF*	<i>Mental Health Assessment Form</i> (SSCD) including Mental State Assessment, also referred to as Mental State Examination (MSE).
MHERL	Mental Health Emergency Response Line
NOCCs	National Outcomes and Casemix Collection (mandatory) (refer to Australian Mental Health Outcomes and Classification Network)
OCP	Office of the Chief Psychiatrist
ODC	On Duty Clinician. Refer to Appendix II ODC Role
PSOLIS/WebPSOLIS	Psychiatric Services Online Information System – web and paper based
RAMP*	<i>Risk Assessment Management Plan</i> (SSCD)
SSCD	State-wide standardised clinical documentation (mandatory)
CTRS	Crisis Triage Rating Scale applied within the triage function in WebPSOLIS to determine urgency for follow up.
WebPAS	Web-based Patient Administration System.

*SSCD

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Procedure details

Services

Youth, Adult, Older Adult and Aboriginal CMHS operate across a variety of EMHS sites including (but not limited to) Bentley, Midland, City East, and Royal Perth Hospital. These CMHS provide specialist, multidisciplinary MH assessment, treatment, and care for consumers who:

- are aged 16 years and older
- reside in the respective catchment areas
- meet the service specific referral criteria.

The CMHS clinicians will practice in accordance with:

- principles of trauma informed care ²
- principles of risk assessment ³
- a recovery approach ⁴
- **Mental Health Act (MHA) 2014**
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- WA Aboriginal Health and Wellbeing Framework 2015-2030.

Referrals

Administrative Assistant/Clerical Support Officer (CSO):

Where applicable the service appoints an Administrative Assistant or Clerical Support Officer (CSO) to support the Triage function. The duties include:

- Referrals may be received through the Administrative Assistant/CSO.
- The Administrative Assistant/CSO notifies Triage clinicians of consumer self-presentations for further advice and/or assessment as required.
- Transfers calls to Triage clinicians requiring clinical advice and/or assessment as required.
- Takes phone messages and sends relevant demographic and context details to Triage clinicians via Triage Referral email when staff unavailable to take calls (not inclusive of urgent calls which are escalated to the next available ATT staff member to take).
- Provides clerical support to Triage in way of:
 - Responding and actioning
 - face-to-face enquiries from Triage
 - email requests from Triage.
 - Create WebPAS chart only to initiate an episode.
 - Collating and preparing referral packs (hard copy referrals that typically come through via email or fax) and delivering this to Triage.
 - Booking required medical appointments for consumers via patient information systems (WebPAS).
 - Facilitating other data entry requirements via patient information systems for clinical Digital Medical Record (DMR) reporting functions.
 - Ordering and maintaining required forms and stationery for Triage use.
 - Other clerical duties as directed by Administrative Assistant or Program Manager in accordance with Triage and service needs.
 - Once a triage is completed and processed through the clinical intake meeting. The status is updated on the Intake Meeting tracker and then updated on PSOLIS by the Administrative Assistant/CSO.

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Triage Officer/Clinician role:

Triage clinicians will screen referral documentation throughout their shift and prioritise referrals based on apparent urgency, risk, and workload planning. The Triage Officer is also responsible for:

- Responding to incoming phone calls.
- Ensuring there is adequate information included in the referral for a triage assessment to be completed and to determine if the consumer meets admission criteria.
- Where insufficient information is provided, the Triage Officer may contact the referrer and/or the consumer to seek additional information.
- Prioritising referrals according to risk and the CTRS.
- Managing the workload of triage ensuring that triages are being responded to in a timely manner according to the CTRS and set Key Performance Indicators. This may include a request for all ATT clinicians on duty to assist and complete triage assessments.
- Providing feedback and plans to the daily clinical intake meeting.
- Communicating with referrers regarding outcome of referrals.
- Adhering to performance and compliance monitoring requirements of EMHS.
- Completing a clinical orientation to the triage role and mandatory triage training.

Initial triage process and contact timeframes

- The triage officer will record the date the referral is received, record the name of the triage clinician on the referral and request a chart only appointment in WebPAS. Where referrals are sent outside of business hours the triage officer will note the date and time that the referral is sent, on the triage.
- Triage clinician reviews for acuity (referral progressed according to acuity gauged from referral info).
- Referral is placed in “in tray” for actioning.
- The triage clinician will attempt to contact the referred consumer/ Personal Support Person (PSP) /referrer as soon as possible, but no later than one business day to initiate the triage process and gather relevant information to complete the triage on WebPSOLIS.
- The WebPSOLIS triage document starts from the first attempted contact. If the triage is unable to be completed it will be marked as “awaiting further information”.
- If initial contact is not successful, the clinician is to complete the WebPSOLIS triage based on the referral information and collateral gathered from PSOLIS/DMR/iCM.
- The outcome on the WebPSOLIS triage is: ‘referred to clinical intake meeting’, the action tab is: “clinical intake – ‘further triage required’”.
- Add referral to ATT/Older Adult spreadsheet.
- On completion of referral complete the Activation Checklist.
- Where applicable, handover to ATT After Hours (AH) team staff to continue to contact consumer/PSP. These contacts are to be recorded as service events in PSOLIS and documented in DMR.
- Discuss at intake meeting.
- Referral stays with triage for: further information gathering, consumer contact, RAMP completion.

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ATT End of day handover:

- Any referrals still in 'referral tray' by day's end are handed over to ATT AH team. AH ATT clinicians are expected to review and triage referrals and contact referred consumer during their shifts when not engaged in urgent clinical care.
- AH team follows the same process as above.

ATT Weekly backlog clearance:

- Any referrals still in the 'in tray' by end of the week are to be distributed across all ATT staff for completion before the weekend. Each site to follow their internal process for this.
- All contact attempts and triage outcomes must be documented appropriately in the consumer's DMR, and the service activity is to be recorded within PSOLIS.

Triage unable to contact/non-attendance

When the triage officer/allocated clinician is unable to contact the referred consumer or PSP within 72 hours, a decision based on the risks and evidence available is then presented by the clinician to clinical intake meeting as to whether:

- A home visit (HV) is indicated to further assess the consumer, including consideration for AH HV.
- Additional contact with PSP is required.
- If the referred consumer is unable to be contacted in 7 days, a letter, email or SMS requesting contact in the next 7 days to be sent.
- The Intake meeting will determine if further attempts to contact the consumer and PSP during the 7-day period is required. Decision making is based on known risks and is documented in the DMR.
- If contact is not established in the 7-day period outlined in the letter, the referral is to be presented to clinical intake meeting and closed.

Community Older Adult Triage and Specialty Service Teams may have minor variations in their local processes; they remain aligned with and inclusive of the core procedures outlined above to ensure consistency in triage practice and documentation.

Triage assessment requirements:

Triage must involve active, two-way communication via one of the following methods:

- Telephone (in accordance with the Staff Use of Electronic Devices in Clinical Areas Policy),
- Video call (e.g., via Health Direct), or
- Face to Face contact.

The purpose of the assessment is to determine:

- Level of acuity,
- Risk, and
- Healthcare needs of the consumer
- Suitability of the referral for service.

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Triage process is not complete until phone or face to face contact is made with the referred consumers or PSP. Reviewing the referral documentation or file alone is **not** considered a triage. The following must be completed as part of the Triage process in WebPSOLIS:

- State-wide Standardised Clinical Documentation (SSCD) RAMP.
- SSCD Triage.

**Initiate downtime procedure if WebPSOLIS is unavailable.*

All assessments, clinical decisions, and contact attempts must be accurately and promptly recorded in the consumer's DMR

Cultural support (e.g., Aboriginal Health Liaison Officers, language interpreters, and other cultural services) must be offered and engaged as clinically indicated throughout care.

Acute response

On completion of the triage assessment the clinician is required to formulate a plan of action as indicated by the CTRS rating (below) and respective timeframes:

Category	Definition
A Immediate	Immediate response requiring WAPOL/ambulance or other service e.g., overdose, siege, imminent violence.
B Within 2 hours	See within 2 hours/present to psychiatric emergency service or emergency department e.g., acute suicidality, threatening violence, acute severe non-recurrent stress.
C Within 12 hours	See within 12 hours e.g., distressed, suicidal ideation of moderate to severe nature, disturbed behaviour.
D Within 48 hours	See within 48 hours e.g., client / carer distress, early symptoms of relapse, person known to the service requiring priority review.
E Within 2 weeks	Non urgent. A routine referral is for a non-urgent assessment.

Referral review and intake:

- Following Triage, all referrals must be presented to clinical intake meeting at the earliest opportunity to ensure timely assessment and prioritisation.
- If an urgent referral is received and triaged over weekend and clinical or medical advice is required, the on-call Psychiatrist can be contacted. The referral and subsequent actions/outcomes can be presented at the next clinical intake meeting.

Documentation and meeting outcomes:

- Referrals will be added to the standardised intake spreadsheet and discussed during the next clinical intake meeting.
- Documentation on this spreadsheet should be minimal and reflect only the referral status and outcomes from the intake meeting.
- Detailed clinical information must be recorded in the DMR and PSOLIS only.

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Cultural and diversity considerations:

- For Aboriginal consumers, consult the aboriginal health teams for access to an Aboriginal Health Liaison Officer (AHLO) if the consumer wishes to involve elders, traditional healers, or other community members in their care.
- Engage cultural support services, language interpreters, and LGBTIQ+ Allies as required.

Triage outcomes

When a referral is triaged, an SSCD Triage must be completed in WebPSOLIS with one of the following outcomes:

1. Unable to Complete

- Referral information is registered in WebPSOLIS.
 - Consumer could not be contacted and further information is required.
 - Client Did Not Engage.

2. Refer To Clinical Intake

- The triage assessment has been completed.
- The referral is presented at the next clinical intake meeting for multidisciplinary review and service planning.

Triaged referrals are presented to the next intake meeting to determine if the consumer meets admission criteria and is accepted to service.

3. No Further Action

Where the referral is not accepted to service the Triage clinician will ensure:

- A letter is provided to the consumer with recommendations.
- A letter to GP/referrer with triage outcomes and recommendations.
- Refer the consumer to appropriate external community services (e.g., Head to Health, Headspace, or others as applicable).

Initial assessment/activation to CMHS

Activation and allocation timeframes

- All referrals must be activated within 14 calendar days of triage, without exception.
- Referrals should be presented to the next clinical intake meeting at the earliest opportunity for allocation to a CC.

* East Metropolitan Eating Disorders Specialty Service model of care provides direction for allocation and activation timeframes specific to their cohort and service delivery structure, this at times includes the introduction of a waitlist that may be contrary to the above directive.

Care coordinator responsibilities upon allocation

CCs must undertake the following actions to ensure safe and effective care coordination:

- Determine and document the consumer's preferred method of communication. If the consumer declines SMS reminders:
 - Use phone calls and/or appointment cards.
 - Advise clerical staff to:
 - Remove the mobile number from the "mobile phone" field in WebPAS,
 - Enter the mobile number into the "home phone" field if needed for clinical use.

Initial assessment

- The allocated CC must establish contact with the consumer within one business day of allocation and arrange an initial face-to-face assessment within 7 days of allocation.
- If this timeframe cannot be met the reason must be clearly documented in the DMR and reviewed at the clinical intake meeting.

At the initial face-to-face meeting, the following must occur:

- The *Personal Support Persons and Consent to Release/Receive Information Form* must be completed with the consumer.
- The consumer must be provided with a Consumer Information Pack, including service details and emergency contact options.

Initial assessment documentation

While Initial assessments will generally be completed by the Care Coordinator at their first face to face assessment, an initial assessment may be completed as part of a medical review with the CC present with both clinicians contributing to the completion of the MHAF. All other SSCD forms are completed by the Care Coordinator. The MHAF, RAMP, TSDP, NOCC measures and other SSCD forms on must be completed on WebPSOLIS within 14 days of allocation. The MHAF should be completed again where there is a change in mental state requiring admission and annually. Other assessment and screening tools may also be used with consumers (e.g., *Alcohol, Smoking and Substance Involvement Screening Test [ASSIST]*, *Nicotine Replacement Therapy [NRT] Chart*, *RUDAS*, *Transcultural MH Assessment*, *Screening for Family and Domestic Violence [FDV] Assessment Tool*, etc.).

Medical review appointment process

Once the MHAF has been completed and presented to the Multidisciplinary Team (MDT), the CC may book a medical review appointment in WebPAS. Medical Review appointments should occur in conjunction with or after initial assessment is completed, to ensure that assessment findings inform the treatment approach. They should attend all medical appointments with the consumer, wherever possible. Where an urgent medical review is required prior to completion of the full assessment:

- The consumer must be presented to the MDT and Medical Officer (MO), and
- An urgent appointment may be arranged. This can be facilitated via the Duty MO where there is one identified within the team structure.

Physical health examination requirements

- The CC must arrange an appointment with a MO to complete the **SSCD Physical Health Examination form** in **WebPSOLIS**. This appointment must be booked **within 30 days of the initial assessment**.
- The Physical Health Examination must be **updated every 6 months** as part of ongoing care.

Communication with GPs

The treating MO must provide a letter to the GP every time a consumer is reviewed. The letter should advise the GP of the consumers, presentation, mental state and management plan.

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ATT transfer to CTT/other teams

For teams where the triage is performed by an ATT, the assessment outcome may recommend the consumer be transferred directly to CTT or another internally located treatment team. Referrals that may fit this criterion are:

- Referral of a consumer who is an active consumer in a CTT from another Community Mental Health Team and will require ongoing CTT support to reach their recovery goals.
- A consumer whom the ATT MDT determine will require ongoing care co-ordination to reach their recovery goals.
- A consumer being transferred on a Community Treatment Order (CTO).
- A consumer who is receiving Clozapine Treatment.

When a consumer is identified in the ATT MDT as suitable for CTT/Other teams follow-up, the ATT or Triage clinician will:

- Register the referral and collate the demographic and Initial Assessment documentation outlined below.
- Contact the consumer to ensure they are living in the catchment area and, if a voluntary consumer, that they are agreeable to engagement.
- Present the referral to the ATT TL within the ATT Daily clinical intake meeting with the completed Care Transfer Summary for active consumers or Triage and RAMP for non-active consumers.

ATT and CTT/other internally located team - team leader handover

On the same day as the transfer meeting, the ATT TL and CTT TL will review each referral and associated clinical documentation including:

- SSCD Triage
- Updated SSCD RAMP
- SSCD Mental Health Assessment
- SSCD Treatment, Support and Discharge Plan (TSDP)
- SSCD Care Transfer Summary
- NOCC Admission measures.

(Note: NOCC Discharge measures must be completed at the time of transfer)

If documentation is incomplete, it will be returned to the ATT intake meeting for re-presentation and planning. The outcome of the ATT–CTT TL discussion must be documented in the DMR by CTT TL.

Presentation to CTT/other team MDT

The relevant team's TL presents the referral to the allocated care coordinator. Once allocated the teams administration assistant/CSO notifies ATT via email and includes:

- Assigned:
 - Care Coordinator,
 - MO,
 - Administration staff for activation and handover tracking.

Effective transfer of care requires a commitment to solution focussed communication between treating teams in a timely manner.

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Non-attendance and disengagement follow-up protocol

All risk assessments must be understood as dynamic and time sensitive. Clinicians must not rely on previous risk assessments (particularly those marked "low risk") to guide their actions when consumers disengage or miss appointments. Clinical decision-making must be based on the consumer's current presentation, level of engagement, and psychosocial circumstances.

When a consumer is not contactable or misses an appointment

Whether following initial allocation or during ongoing care, if a consumer cannot be contacted or does not attend a scheduled appointment (clinic, home, or community):

- The treating clinician (with whom the appointment was made) must attempt direct contact on the same day the consumer fails to attend/engage.
- If contact is unsuccessful, the CC should be informed so that they can contact the nominated PSP, to determine the consumer's whereabouts and well-being. Where there are concerns about the consumer, the CC does not require the consumers consent to contact PSP.
- All contact attempts must be clearly documented in the DMR and WebPSOLIS.

Escalation to MDT review

If the consumer cannot be contacted within 1 business day of allocation or has not been seen within the minimum required clinical frequency (2 weeks by default, or 4 weeks if endorsed by the MDT), the CC must present the case to the MDT to:

- Re-evaluate the initial or prior risk assessment, noting that risk is dynamic and may have changed.
- Assess current clinical risk. Where risk is heightened or uncertain, plan for a home visit or other appropriate outreach.
- Develop a plan for engagement, follow-up, or service withdrawal.
- Determine whether the referral should remain open or be closed based on clinical judgment and consumer need.

If the referral is to be closed

Where the MDT recommends closure of the referral:

- The CC must document the outcome and clinical rationale in the DMR.
- The consumer, their support person, and the referrer must be notified via phone, email, or letter.
- The consumer must be provided with appropriate mental health emergency contacts and instructions on how to re-access services if required.

Documentation

- Risk-related decisions and follow-up plans must be reflected in an updated Risk Assessment and Management Plan.
- Interventions may include: increased outreach, home visits, welfare checks, escalation to AH services, or other tailored strategies.
- All MDT discussions, clinical decisions, and follow-up actions must be documented in the health record and updated in WebPSOLIS, including within the TSDP.

Screening requirements

Ensure all CMHS consumers are screened on presentation for Respiratory viruses as per the CMHS Responding to Respiratory Virus Illness SOP.

Home visit protocols

- If assessment occurs via home visit, follow the Mental Health Community Home Visiting SOP.

Commencement of discharge planning

Discharge planning must begin at the point of entry to the service and be documented progressively within the TSDP. This plan should reflect both short- and longer-term goals and be regularly reviewed in collaboration with the consumer and their support network. All information provided must be clearly documented in the DMR.

Reviews and ongoing care

CCs must maintain face-to-face contact every 1-2 weeks. If less frequent contact is clinically appropriate:

- This must be evidenced by:
 - low risk assessment within the RAMP.
 - the MDT, including the TL and Senior Medical Lead, and
- Documented in the DMR and the TSDP.

The frequency of contact required is to be reviewed at every 90 day clinical review.

Cultural support (e.g., Aboriginal Health Liaison Officers, language interpreters, and other cultural services) must be offered and engaged as clinically indicated throughout care.

Documentation requirements

No entry should be made into the medical records more than 72 hours after the day of service. All service delivery must be recorded in:

- DMR as soon as practicable. If recorded after the day of service, the entry must be clearly marked 'late entry'.
- PSOLIS/WebPSOLIS as soon as practicable.

Clinical review templates

Clinical review templates will include the following:

- Attendees, consumer's current clinical presentation, issues, risks, plan for ongoing care, phase of care, frequency of contact, stakeholders/supports, legal status, discharge barriers and outcome of MDT.
- Dates of most recent SSCD reviews:
- RAMP
- TSDP or Stay Strong Plan, Crisis Plan, or Safety Plan, including Predicted Date of Discharge
- PSOLIS alerts reviewed, revised or expired as needed
- NOCC measures
- Last and next medical review
- GP letter

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- Physical Health Examination (required every 6 months or more often if clinically indicated),
- Include use of relevant screening tools:
- Metabolic Screening:
 - Clozapine (refer to Clozapine Monitoring Form Part B or C),
 - Lithium (refer to Lithium Monitoring Form)
 - Additional screening tools used (e.g., RUDAS, MoCA, ASSIST, FDV Assessment Tool)
- MHAf completed annually

Requirements for Care Coordination checklist must be completed in each Clinical Review.

Clinical review timelines

ATT Team Clinical Reviews are conducted at:

- 4 weeks,
- 8 weeks, and
- 10 weeks (discharge planning).

These reviews are prepared in accordance with approved clinical review template and presented to the MDT.

All other Community Mental Health Reviews are conducted at:

- Activation
- 3 monthly
- Discharge
- Change in presentation/plan.

These reviews are prepared in accordance with approved clinical review template and presented to the MDT.

Medical review requirements

All medical reviews must comply with the RPBG Minimum Requirement for Medical Review of Consumers Admitted to Inpatient Wards-or CMHS Policy. Emergency or urgent reviews must be discussed via the Duty MO/Treating Doctor, as clinically appropriate.

For prescribing, medication storage, and related practices, refer to the RPBG Prescribing Policy, and Medication Management and Storage Clinical Guidelines.

The treating doctor must send a letter to the consumer's GP after every clinic review. This should inform the GP about the consumers presentation, mental state, risks and management plan.

Escalation of care

Where a consumer's mental health deteriorates, the CC (or clinician) and Treating Doctor should work with the consumer to determine suitable actions to manage their mental state and risk. Where there is a heightened risk the CC and MO is to escalate the matter to the Consultant Psychiatrist and Team Leader.

If the MHA 2014 needs to be utilised, a review with the treating Doctor or an Authorised Mental Health Practitioner should be facilitated with the consumer. MHA forms may be utilised to direct the consumer to an authorised setting for further assessment. Where this is deemed necessary by the authorised persons, they should:

- Liaise with EMHS mental health patient bed flow re available inpatient beds and provide handover
- Treating Doctor / CC to liaise with relevant hospital site's Emergency Department
- Enter consumer details on Enterprise Bed Management system for RPH and Bentley inpatient requests
- Complete the MHAF or Triage Form (for ATT) and RAMP (and physical assessment where possible) and provide to the authorised setting.
- Arrange for patient transport
- Ensure consumer, clinician and PSP safety.

If the consumer becomes aggressive and attempts to leave, Mental Health Service staff should not attempt to prevent the consumer from leaving. The safety of staff and other consumers must be a priority. If the consumer leaves, the Transport Order is to be referred to WAPOL with a risk assessment (using WAPOL forms). The TL and Consultant Psychiatrist should be advised and the Program Manager and Executive Team are to be informed if an incident occurs.

For clinical escalation, refer to:

- **Recognising and Responding to Acute Deterioration (RRAD) in Mental State, Cognition and Behaviour and/or RRAD Policy**
- [Appendix I: Managing increasing risk relating to consumers awaiting transport for transfer from BHS CMHS to ED/non-BHS MH inpatient unit](#)
- [Appendix II: Managing increasing risk relating to consumers awaiting transport for transfer from Midland, City East and Wungen Kartup CMHS to ED/off site MH inpatient unit.](#)

Intra team handover

If a CC is to be on extended unplanned leave, the TL /Senior clinician is to review the clinician's caseload including DMR and care coordination dashboard and reallocate consumers to other clinicians within the team. CCs should provide a formalised clinical handover of consumers when they are on leave for 2 or more weeks or when they are leaving the service temporarily (due to secondment) or permanently. For clinical handover, CCs are to:

- Ensure relevant consumer SSCDs that are due for review within 2 weeks of commencement of leave are renewed prior to leave
- Provide face to face or telephone handover to the clinician(s) covering their caseload, identifying consumers requiring active follow up during the leave period
- Provide a written handover using the PSOLIS Clinical Handover Sheet. (refer to relevant PSOLIS training).

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Discharge process

Prior to discharge a Discharge planning meeting should be held involving the consumer/ PSP, future care providers and any other key stakeholders (including the consumers GP supporting the consumers recovery. The CC should discuss with the consumer or PSP the process for re-entry to service and contacts with crisis services. The CC should also seek feedback re the quality of service provided and areas for improvement from consumer and/or PSP. If a discharge meeting is unable to occur, evidence of attempt and reasons for non-occurrence must be documented in the consumer's DMR

At discharge a deactivation clinical review must be held with the MDT including a review of the following:

- CTS (SSCD):
- discharge letter to GP or receiving service
- TSDP or Stay Strong Plan with Crisis Plan or Safety Plan
- Complete NOCC measures
- Deactivation RAMP where applicable Community Treatment Order and *MHA 2014 forms* should be completed and transferred to another CMHS.

The treating MO is to arrange medications and prescriptions at least 24 hours prior to discharge and liaise with Pharmacy where necessary (e.g. organising a webster pack).

On closure of the DMR the CC is too complete *PSOLIS Deactivation NOCC* and administration assistant/CSO input the Referral Outcome and close the WebPAS episode.

Compliance monitoring

Compliance with this procedure will be monitored:

- During clinical incident investigations via Datix Clinical Information management system (CIMS) and/or Combined Hazard or Incident Reporting (CHoIR) Online Form or [Notifiable Incident Reporting to the Office of the Chief Psychiatrist](#), if appropriate
- RPBG Performance Management Audit – Compliance with the SOP will be measured through the Triage to Discharge audit against the DMR
- Complaints via the Consumer Feedback Module.

Authors / acknowledgements

We acknowledge the following previous site endorsed work and/or contributors used to compile this document.

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Community Mental Health Services Triage to Discharge (Youth, Adult and Older Adult) SOP through consultation with the Aboriginal Health Strategy Unit and Wungen Kartup to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 1596).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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References

1. What is Trauma Informed Care, University at Buffalo, Buffalo Centre for Social Research, The Institute on Trauma and Trauma-Informed Care. Accessed 16/09/2020 <http://socialwork.buffalo.edu/social-research/institutes-centers/institute-on-trauma-and-trauma-informed-care/what-is-trauma-informed-care.html>
2. Commonwealth of Australia. (2013). *Department of Health National Framework for Recovery Oriented s: Guide for practitioners and providers*. [National Framework for Recovery Oriented MH Services: Guide for practitioners and providers](#)

Related policies, practice standards, clinical guidelines

Policies and Procedures SharePoint

- [Infection Prevention Management \(IPM\) policy set](#)
- [Assessing Patient Capacity SOP](#)
- [Community Mental Health Home Visiting SOP](#)
- [Clinical Handover Policy](#)
- [RPBG MHD HealthPoint page](#)
- [Staff Use of Electronic Devices in Clinical Areas Policy](#)
- [Community Mental Health Service \(CMHS\) Provision: Responding to Respiratory Virus Illness SOP](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State, Cognition](#)
- [RRAD Policy](#)
- [Unplanned Presentations to BHS SOP](#)
- [Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy](#)
- [Alcohol and Drug Intoxication and Withdrawal Management Clinical Guideline](#)

EMHS Policies

- [Care Coordination in Mental Health Services Policy](#)

WA Department of Health Policy Hub

- [Mental Health Status Assessments: Role of Mental Health Clinicians Policy](#)
- [Clinical Incident Management \(CIM\) Policy](#)
- [Clinical Handover Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Psychiatric Services Online Information System \(PSOLIS\) Mental Health Act \(MHA\) 2014 Business Operating Rules](#)
- [Mental Health Information Data Collection Manual](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services Policy](#)
- [SSCD for Mental Health Services Procedure](#)
- [Triage to Discharge: Mental Health Framework for SSCD](#)
- [WA Aboriginal Health and Wellbeing Framework 2015-2030](#)

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Other

- [Mental Health Act \(MHA\) 2014](#)
- [Chief Psychiatrist's Standards for Clinical Care \(2015\)](#)
- [Notifiable Incident Reporting to the Office of the Chief Psychiatrist](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [National framework for recovery oriented mental health services: Guide for practitioners and providers – Department of Health](#)
- [Australian Mental Health Outcomes and Classification Network - MH NOCC Technical Specifications](#)
- [Australian Mental Health Care Classification \(AMHCC User Manual\) – Independent Health and Aged Care Pricing Authority \(IHACPA\)](#)

Related forms

- *Community Mental Health Care Coordination forms:*
 - 1. *Triage-Activation Checklist*
 - 2. *Discharge Checklist*
- *Consumer Registration Form*
- *Personal Support Persons and Consent to Release/Receive Information Form*
- *Montreal Cognitive Assessment (MoCA)*
- *Alcohol, Smoking & Substance Involvement Screening Test (ASSIST)*
- *Mental Health Transcultural Assessment*
- *Screening for Family and Domestic Violence (FDV) Assessment Tool*
- *Clozapine Monitoring Continuation Form B or C*
- *Lithium Monitoring Form (Mental Health)*

Legislative requirements, the evidence and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy HUB.

ACSQHC NSQHS Standards 2nd Edition (2021)

1. Clinical Governance
2. Partnering with Consumers
6. Communicating for Safety
8. Recognising and Responding to Acute Deterioration

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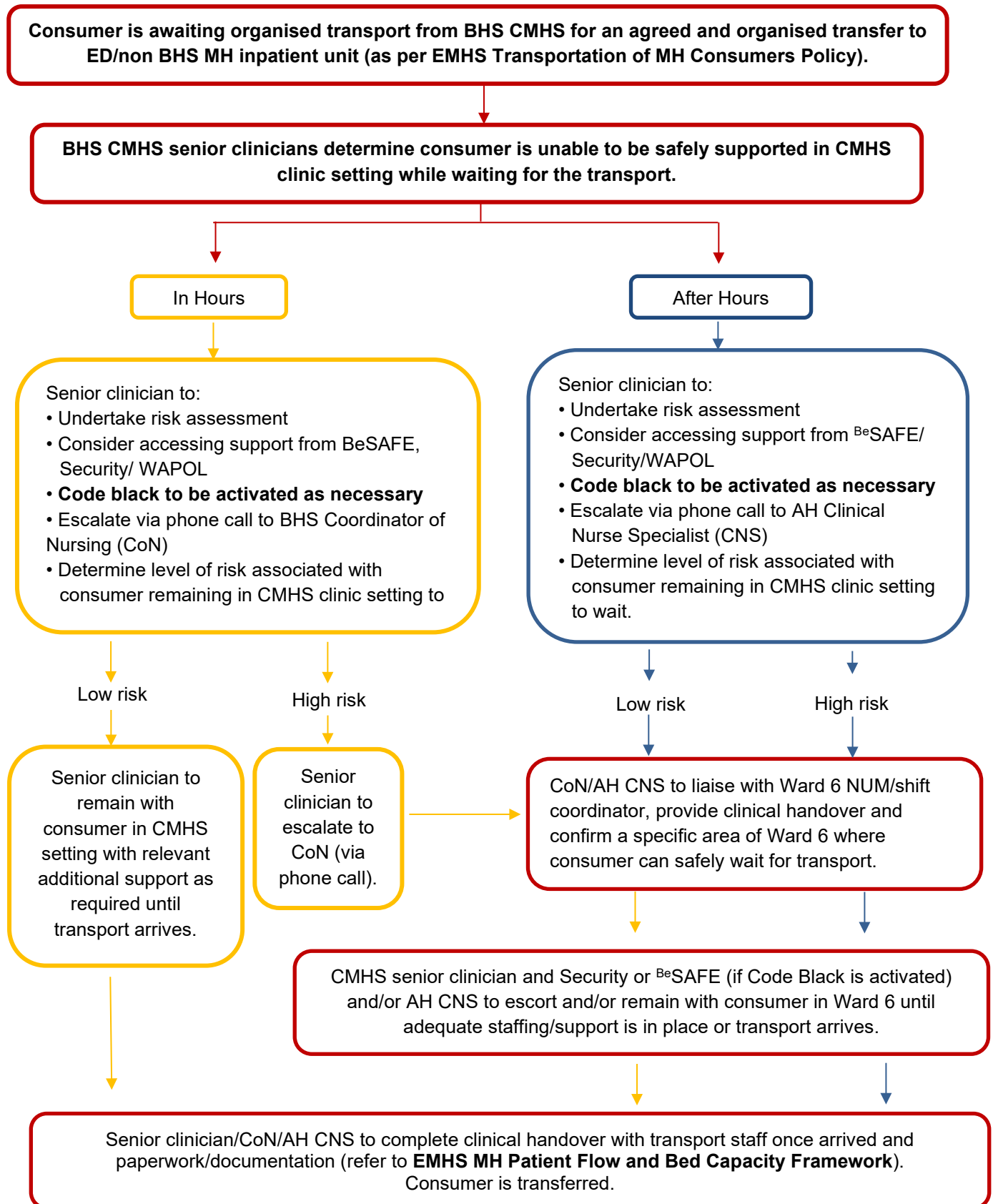
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Appendix I: Managing increasing risk relating to consumers awaiting transport for transfer from BHS CMHS to ED/non-BHS MH inpatient unit



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Appendix II: Managing increasing risk relating to consumers awaiting transport for transfer from Midland, City East or Wungen Kartup CMHS to ED or an offsite MH inpatient unit

Consumer is awaiting organised transport from YCATT, EMEDSS, Midland, City East CMHS for an agreed and organised transfer to ED or off site MH inpatient unit (as per **EMHS Transportation of Mental Health Consumers Policy**).

CMHS senior clinicians determine consumer is unable to be safely supported in CMHS clinic setting while waiting for the transport.

Senior clinician responsibilities:

- Call an ambulance for a medical response and remain with the consumer where safe to do so, until transfer occurs
- Where it is unsafe to wait with the consumer, close waiting area or meeting space and contain within where practicable
- Follow local emergency procedures
- Consider the need for Security (where available) and WAPOL attendance as appropriate
- Use risk mitigation strategies to support the consumer to safely remain in situ
 - Within the clinic setting - confirm a specific area where consumer can safely wait for transport.
 - Outside of a clinic setting - (e.g., Wungen Kartup), refer to **RPMG Community Mental Health Home Visiting SOP**.
- Complete documentation including clinical handover with transport staff once arrived
- Also refer to **EMHS Mental Health Patient Flow and Bed Capacity Framework**.

On Duty Clinician (ODC) role

- When the allocated CC or MO are unavailable or either of these has not been allocated, ensure that active or new MH consumers gain access to advice, treatment and care by **making contact with the consumer within one hour**
- The ODC for all services can be any appropriately qualified and experienced mental health professional. The ODC within
 - Youth and ACMHS:
 - Midland, Bentley and City East – is the rostered, allocated clinician
 - Wungen Kartup – is the Triage Brief Intervention Officer
 - OACMHS – is the Triage Officer
- Administration Assistant /CSO *do not* include assessment of urgency of the call. All calls should be handed over to the consumer's CC or TL or Senior Clinician or ODC.

Initial contact

- When an active or new consumer presents (face to face or via phone) to a CMHS outside of a regular appointment, clerical staff are to initially
- Assist with identifying the consumer's registration details and next medical appointment by searching WebPAS or PSOLIS to identify associated CC

- Determine if the consumer's CC or MO is unavailable or there is no allocated CC, and with the consumer's verbal consent, refer them to the relevant ODC or CTT
- Notify the consumer's CC and ODC of outcome, including if a consumer terminates the call or presentation prior to completing a referral to their CC or ODC
 - If the CC is unavailable and the ODC is busy for an extensive time, refer the consumer to the relevant Senior Clinician or TL

The ODC is to:

- Re-confirm the consumer's registration details, including the allocated CC and MO and access the consumer's progress notes for background information:
 - If the consumer is unable to identify their CC or MO, search via WebPSOLIS
- Ascertain the reason for the consumer's presentation or phone call and/or check consumer's next medical appointment via iCM /DMR or with their CC or MO
- Based on the reason for the consumer's presentation or call, determine if the consumer is:
 - Able to wait until this appointment **or** until the allocated CC or MO can return the consumer's call

OR

 - In need of immediate assistance
- Complete a RAMP if indicated and refer to Escalation of care section
- Responsible for the consumer's care, i.e. provide immediate and short-term intervention services and/or information, e.g. referral to a dentist or counselling
- Liaise with the consumer's PSP, if appropriate
- Liaise with the ATT during AH (1630 – 2200 hours), including weekends and public holidays (0830 to 2200 hours), if required
- Complete clinical handover between shifts (iSoBAR handover), including communication, actions taken and/or appropriate follow up with consumer's CC or treating team and/or with ATT (for Bentley CMHS)
- Document all contacts with the consumer, assessments, management or crisis plans and the outcome of actions taken via WebPSOLIS and in the consumer's DMR and complete the ODC Report (circulated to team members at end of shift)
- Other ODC duties include to
- Respond in a timely manner to phone calls from the general public and health professionals seeking MH information and provide appropriate advice
- Additional tasks for the City East CMHS ODC only are as follows:
 - At 1615 hours contact any clinicians who are on a scheduled home visit and have not yet returned or contact the CMHS to advise of the clinicians return and safety or are otherwise accounted for.
 - Check the duty diary and organise completion of any duties and requests that are listed.

After hours consumer contact with CMHS

Active consumers who contact the Adult/Youth/Older Adult CMHS AH, including weekends and public holidays - calls are redirected via answering machine message or switchboard or clerical staff to MHERL, after 1630 hours. MHERL will redirect calls to ATT as required.

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